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The AD literature seems to go from miracle to failed miracle almost daily with little attempt to integrate new results with old observations. There are now a lot of data points to put together with cause and effect like any TV detective would do or as described in fables like the blind men and the elephant even though apparently Natalie Merchant set it to music. This work tries to fix that and finds that all these things are logically explained by well known issues such as age related digestive defects. Suggestions are further supported by in-house work on optimization of dog diets demonstrating apparent safety and utility of out of favor vitamins that could be impacted by changes in stomach chemistry. Another problem may be politics. One issue may be related to tyrosine metabolism and increased relative abundance of analogs capable of becoming toxic. This immediately jumps out as a contributor to race based AD differences not just social aspects that are touted as they are politically correct. To be sure, constitutive tyrosine metabolizers may have adaptive responses which support this flux with no net "bad" effects elsewhere and there may turn out not to be any anyway. In any case, its an important issue and like any other law of nature will not yield to human desire and the only way for people to benefit is follow the data. If you want to fool society, appease angry people, instead of solving problems and eliminating run on sentences for everyone turn back now to your safe space... **This is a draft and has not been peer reviewed or completely proof read but released in some state where it seems worthwhile given time or other constraints. Typographical errors are quite likely particularly in manually entered numbers. This work may include output from software which has not been fully debugged. For information only, not for use for any particular purpose see fuller disclaimers in the text. Caveat Emptor.**

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Alzheimer's Disease : 10k maniacs agree its an elephant

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(Dated: March 17, 2026)

Effective treatments for Alzheimer's Disease(AD) have been difficult to create. Much effort has been directed at amyloid beta removal with minimal clinical benefits demonstrated. Several alternative approaches have become popular including the infection hypothesis. Since so many pathogens appear to cause AD an age related vulnerability may be a better focus. Over various time spans, a series of headlines has emerged claiming benefits for various nutrients or supplements such as thiamine, choline, lithium, or NAD+. However, none appear to generate robust recovery. Taking these results and other observations together, similar to the fable of the blind men grasping different part of an elephant, a plausible unifying theme is nutrient deficiency due to age related digestive defects. Isolated profound nutrient deficiencies will not likely exist so obvious attribution of disease to single molecular entity is not a good starting point. Note that this goes beyond failure to absorb nutrients but also the creation of age related toxins, possibly methanol for example, that may need to be mitigated. Adaptive and maladaptive responses may make such an initial cause difficult to discern. The literature on the gut-brain axis addresses some of these issues but does not try to identify specific causes for GI disturbances. This work motivates "meal engineering" as an intervention strategy that may be able to correct many of these problems. If this analysis accurately captures cause and effect in the real disease, correction or prevention may be fairly easy with well known small nutrient molecules or addition of acid or chloride to meals. Individual responses to nutrient details

will appear to make each case different but still possibly treatable with similar simple remedies. Indeed, there has been some limited success with dietary interventions but these fail to be designed around likely age related causes although they may coincidentally mitigate some of these problems. It may not be possible get all required nutrients from food once damage to digestive system has occurred and supplements or other entities may be required. Cations such as metals, protein bound nutrients, amino acids, and lipophilic nutrients would likely be the first suspects. In particular, its likely that vitamin K and copper, along with more accepted components such as amino acids and SMVT substrates all need to be included to replace the younger absorption profile. However, brute force supplementation without regard to formulation may not work due to chemical and metabolic interactions. Two important amino acids with solubility issues in particular, tyrosine and tryptophan, and critical but reactive metals such as copper may require special attention. There are also possible political concerns if there is a hesitancy to link tyrosine metabolism to dementia. However, as always, social influences will not change natural phenomena.

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I. INTRODUCTION

Alzheimer's disease [44] remains an unresolved problem with important impact to society [37] despite all the money and effort directed at it for the last several decades [51]. Anti-amyloid products have been the major focus but continue to show questionable net clinical benefit [185] [165] and do not always get FDA approval even with creative trial design [67]. This is in spite of the fact that as early as 2002 some had noted too many inconsistent observations about amyloid or tau for them to be serious targets [157]. Several other approaches and theories have been developed with various levels of support although it would be difficult to say that a specific next-best has emerged. The thesis of this work is that all these distinct approaches can be resolved by looking upstream for initial causes for these diverse observations about the disease. Before detailing these "pieces of the proverbial elephant", some indication of the problem may be appreciated with recent effort to employ AI against AD. Given the different aspects of AD, often described as "multifactorial" or "heterogeneous" emerging AI has been proposed to lead a response [96]. [72]. This may yield results, its progressing too rapidly to rule out, but often it is limited by training not on raw data but human commentary and a statistical rather than "model of reality" based view. Others are using AI for precision medicine thinking that patient heterogeneity is a problem [220]. Some are searching for connections between apparently unrelated observations [83] to find tractable intervention strategies similar to the present work. Headlines sometimes appear about new scientific or anecdotal evidence pointing to a new treatment but rarely do these translate into useful therapies. Part of the thesis of this work is that many of these results are useful but tend to be interpreted in isolation and without regard for cause and effect in real disease despite often intricate and detailed biochemical analyses related to each piece. While some pathway details have been described, the search backward to an "initial" cause is not common. Often the actual cause is treated as irrelevant. "Animal models" may mimic some features of real disease but are predicated on a cause unlikely to be relevant to the real disease [172] and their limitation are becoming more appreciated. By coincidence, sometimes however results may be transferable but don't seem to add to understanding. These will be discussed in more detail in the context of each piece.

The following subsections list some AD related theories or data that motivate the fuller thesis of this work identifying a more or less tractable point of failure which leads to diverse symptoms but yet may be easy to treat. In essence, this work suggests that the best intervention is in the GI tract, similar to dysbiosis theories and malnutrition and diet theories, but with the goal of addressing specific causes of these consequent conditions.

A quick note on taxonomy and focus may be helpful. Dementia in general and AD in particular are not well understood but can be defined by a few concepts. AD is generally defined by amyloid and tau markers [106] but as mentioned later there is reason to believe these have various reasons to occur. Typically AD is divided into a sporadic and familial or late-onset (LOAD) and early-onset (EOAD) type with work continuing to explore differences [176] [27] [221]. This work is generally oriented towards late onset sporadic AD although its likely to address many age related issues too as nutrient effects are not entirely brain specific. Indeed sarcopenia will be noted as a correlate or age and disease. The implicit feature of AD assumed throughout is that it is a disease of older age and theories need to explain that age dependence. The remaining introduction sections discuss classes of observations or data, "pieces of the elephant", that the present work claims to organize into diverse manifestations of age related GI impairment as detailed later.

I.1. Some stumbling blocks

Thinking aloud

on the one hand, this doesn't fully capture my disgust but its also too distracting as written ROFL

The hypothesis presented here largely builds on prior work rather than contradicting many of the details but there are a few exceptions to that. When proposing a new theory that is inconsistent with current thinking, and prior work is stalled, it may be that there are fundamental fallacies in the existing accepted analysis or data. To make the new theory credible it may be important to explicitly identify hidden fallacies or errors in the status quo. Some common ones were enumerated in prior work [138] I listed several common reasons for dead end thinking as the literature appeared to suffer from many. Here too many of those apply and the fixation on amyloid beta, treating numbers to be more normal, ignores upstream cause and effect. AD however is also apparently restricted by social factors as intelligence and behavioral issues may elicit more rage than reason from sponsors. Comforting assertions and assumptions, sometimes to gain more popularity, may be held well beyond what is justified by any data. In other cases, cause and effect may be related in implausible ways which corrupts clear thinking. For example, a recent work looking at sex and race as factors in AD suggests that "educational level" a modifiable risk factor [160]. That may be true to some extent but realistically some may have preexisting brain disfunction making it essentially impossible to achieve meaningful higher education. And we are already seeing unfortunate consequences of thinking a degress

itself, without real accomplishment means anything with the distorted job market [61] [69]. Or, maybe all we need to do is give AD patients a PhD in physics.

I.2. Accumulated Damage Theories

One general class of competing ideas involves "accumulated damage" theories where some cellular components develop non-specific irreparable damage leading to the decline of the brain and body. These can equate chronological age with "wearing out" or invoke something like senescence [16] and just accept "it happens." These may concentrate on vasculature and boarder on hopelessness [11] or ROS damaged proteins [131]. Accumulated genetic damage [236] could include a related group of theories. It may include epigenetic changes [141] , somatic mutations and mosaicism [126] , and telomere shortening [118] . This work addresses the protein quality issues directly and epigenetic changes tend to be programmed responses to things like nutritional status so they are expected to improve too. One work concerned with epigenetic factors recommended supplements of things like B vitamins and s-adenosyl-L-methionine [184]. Telomere length is not directly considered although it correlates with age some associations with AD are expected and a causal role is being explored [49] . Some correlations between length and cognitive decline appear backwards [100]. A large number of nutrients have various correlations with telomere length such as the usual B-vitamins [167] as well as diets [197] such as the Mediterranean [73]. There also appear to be associations with vitamin K intake [52].

Genetic mosaicism is a normal part of the brain as mutations accumulate from conception [21]. Many of the pathological mutations linked to AD are thought to be encouraged by well known AD risk factors [101] suggesting they can be reduced by addressing these known factors. Stabilization or removal of deleterious somatic mutations may be less obvious. Certainly generic defects in DNA repair may not be remediated solely by diet but defect production and culling of mutants may be possible.

I.3. Genetic Causes

Ultimately all disease is genetic since perfect genes would anticipate and deal with everything that can occur but realistically these consider less common mutations that differ from the larger population. The genetic damage theories have some appeal because several gene variants have been associated with AD and appear in pathways that are plausibly causal although this may create confirmation bias in "relevant" gene selection. One recent work notes hundreds of genes have been implicated and points out that amyloid beta could be a response to stimuli such as, " including infectious agents (herpes simplex, chlamydia pneumonia, and Borrelia burgdorferi) as well as Vitamin A deficiency, hypercholesterolaemia, hyperhomocysteinaemia or folate deficiency, oestrogen depletion, cerebral nerve growth factor (NGF) deprivation, diabetes, cerebral hypoperfusion (leading to hypoxia and hypoglycaemia) " [34]. Then the above concentrates on immune related genes. As "genes" are nominally constant through out life, their roles in age related disease may be indirect such as through reduced DNA repair abilities allowing for damage to accumulate faster with age. For the present theory, its worth noting that no popular GI related gene variants have emerged.

Genetics have been widely explored with several common mutations linked to either familial AD(PSEN1,PSEN2, APP) or sporadic late onset (APOE) disease but with many others possibly involved [12] . Among these are things like SLC10A2 and a variety of lipid, calcium, or immune related genes. APOE is associated with many vitamins including vitamin K. APOE was identified as important for vitamin K transport to bones as early as 1996 with apoe4 noted for association with lower serum K and increased fracture risk [116] and it was noted response to supplementation occurs over months. However, a later work found "superior vitamin K status is associated with the apoE4 genotype in healthy older individuals from China and the UK" [226] .

Thinking aloud

find the work on secretase and hair color for example

A some human mutations identified in specific populations, along with humanized genes, have been used to create animal models of AD. The 5xFAD mouse overexpresses mutations in APP and MSEN1 and mimics some features of real disease although not NFT's [162]. EFAD mice include a human APOE gene [207] but still interpretation of any intervention has to be tempered with an understanding of the assumptions inherent in these models. Interestingly, the 5xFAD mice respond differently to diet than wild type. One study found some apparent decoupling of results,

Thus, 5XFAD mice exhibited greater susceptibility to highfat diet than wildtype mice regarding cerebrovascular injury and cognitive impairment. On the other hand, 5XFAD mice fed highfat diet exhibited much less increase in body weight, white adipose tissue weight, and adipocyte size than their wildtype counterparts. Highfat diet significantly impaired glucose tolerance in wildtype mice but not in 5XFAD

mice. Thus, 5XFAD mice had much less susceptibility to highfatdietinduced metabolic disorders than wildtype mice. [129].

These models were largely designed with the assumption that accumulation of amyloid beta was the central feature to replicate but that may turn out to not be correct. Since the cause and effect in real disease would only be captured by luck there could be a lot of limitations from any of these models. Interestingly, transgenic mice may show intestinal symptoms correlating with brain pathology [135].

AD risk varies significantly by sex and race [128] making the related genes suspects. However, other factors such as sociocultural [68] "unique social and environmental pressures" [153] are often blamed. Correlates of sex such as estrogen production may be a more direct factor [180].

Some polymorphisms of the digestive tract are thought to significantly alter drug disposition and they can be race related [75].

More generally, there could be important race or sex based differences in many catagories that are due to easy to treat malnutrition and a focus on social factors distracts from a real solution. Vitamin A related polymorphisms were found to correlate with ethnic group of vitamin A status in a sample of pregnant women [205]. One study of DIO2 polymorphisms and AD among various groups demonstrated a possible contributor to AD in blacks along with a lot of transcription data [147] that may be a good starting point for further hypotheses.

In fact, more directly relevant genes have been found in the black US population. A dopamine receptor D2 variant (DRD2 Taq1AA1) more common in blacks than whites likely associates with AD risk [24]. Presumably variants would differ in some binding kinetics including to dopamin analogs that could become important as parent neurotransmitter becomes more scarce with an aging difective tract.

I.4. Clues from Transcriptional Associations

Besides mutation data, transcriptome analysis has found genes which are over or under transcribed in Alzheimer's Disease. As with most data classes, this has not created an unambiguous path to an intervention and cause and effect, good or bad, remains unresolved for most of these genes and their products. A gene may be over or undertranscribed depending on how it is regulated. Functional feedback could signal for more of a dysfunctional product to be created or one that can not function for some system related reason. So, typical interpretations may be missing important bpoints that may fit well into the model proposed herein.

A 2025 article found ADAMTS2 to be one of the genes robustly increased in black and European Alzheimer's brains [211]. Essentially this translates into pervasive "weak structural proteins" as described below. Mice with non-functional ADAMTS-2 develop fragile skin [125]. It is over expressed in mice and humans with hypertrophic hearts and this condition is worse if ADAMTS-2 is lacking [218] The information so far suggests it is upregulated in cases where is may be beneficial but is unable to accomplish whatever the feedback is regulating. In the case of structural proteins, something may be signalling " more or better " but another factor is lacking (ie malnutrition).

A 2021 work described a liver-brain axis and implicated junk food consumption, perhaps decomposed into both nutrient and toxin exposure, as a driver of AD with related changes in expression of genes such as, " TUBB5-HYOU1-SDF2L1-DECR1- CDH1-EGFR-SKP2-SOD1-IRAK1-FOXO1" and correspondingly, " [gene signature in the] decline of concurrent liver and brain health. Dysregulated levels of these genes are linked to molecular processes like cellular senescence, hypoxia, glutathione synthesis, amino acid modification, increased nitrogen content, synthesis of BCAAs, cholesterol biosynthesis, steroid hormone signalling and VEGF pathway." [93].

I.5. Infection Hypothesis including Cumulative Damage

Another approach gaining interest is the infectious hypothesis of AD [84] [28] [148] [31] . The basic theory is that one or more pathogens cause AD although many are very common such as P gingivalis [80] , varicella zoster [30] , COVID-19 [15] , the Lyme pathogen [84], and Chlamydia pneumoniae [36]. With one abstract listing several diverse contributors [170],

viruses (e.g., Herpes simplex type 1, 2, 6A/B; human cytomegalovirus, Epstein-Barr virus, hepatitis C virus, influenza virus, and severe acute respiratory syndrome coronavirus 2, SARS-CoV-2), bacteria (e.g., Treponema pallidum, Borrelia burgdorferi, Chlamydia pneumoniae, Porphyromonas gingivalis, Prevotella intermedia, Tannerella forsythia, Fusobacterium nucleatum, Aggregatibacter actinomycetemcomitans, Eikenella corrodens, Treponema denticola, and Helicobacter pylori), as well as eukaryotic unicellular parasites (e.g., Toxoplasma gondii) may factor into cognitive decline within the context of AD.

Part of the motivation is the lack of a clear consistent pathology related to amyloid beta but an apparent role in the brain's immune response [91] (which also lists many organisms and advocates infection control) [74]. It has also been observed that H pylori infection associates with prevalence of AD [166], an observation considered by the present work to indicate a related vulnerability is causal to both. Neurosyphilis can be difficult to distinguish from AD [151] creating precedent for at least and AD mimic.

In further support of this , a wide range of vaccines has been found to reduce risk of AD or other dementia [133]. Another study found benefits of vaccination and also anti viral treatment for herpes zoster [140]. There is also some indication AD may be transmissible similar to prion diseases [191].

Thinking aloud

microcyline and stroke could be stopping hypoxic virulence from unknown pathogen

The observed age dependence of AD compared to life long exposure to pathogens has to be reconciled with something like a cumulative damage scenario or an age-dependent host response. Many pathogens cause known GI damage that is often ignored. Covid-19 GI damage and interaction with ACE2 receptors has been studied [142] and secondary bacterial infection with gut organisms is common [19]. Varicella zoster similarly can cause GI issues [156].

Some organisms trigger an interferon response leading to the loss of nutrients like tryptophan [232]. Nutritional immunity can be invoked [158] towards various nutrients with particular emphasis on some minerals like copper [42] that may redistribute in ways that become pathological if deficient. While amyloid may have prion like properties, this kind of transmission does not seem likely in the vast majority of cases. Hypermetabolism has been described in Covid 19 although perhaps underestimated [29] .

I.6. Gut-Brain Axis Hypotheses

Another general group of hypotheses involve the gut-brain axis [94] [97] [124] [229] [169] . Typically this points to something such as the microbiome that gets "out of balance" and needs to be adjusted with inoculation or some feedstock mix or fecal transplant. Often nutrition is considered and a detailed analysis of cause and effect between gut and brain is performed. This may lead to similar nutrients as those considered here but doesn't usually get to the initial cause hypothesized here nor design a solution around that cause.

I.7. Metabolic or Nutrient Hypotheses

This work will consider many combinations of nutrients that have previously been identified as possibly useful in AD. However, these prior works generally don't try to determine a cause or a reason to single out one or a few nutrients.

Works cataloging the utility of vitamins or derivatives for AD exist [214] [194] but generally treat these things as "one at a time" approaches. Just this year a work suggests combining the usual suspects A,B,x,D,E,K [155] apparently neglecting protein and minerals and without real rationale.

Although at least one review from 2022 recognized the likely need for multiple components [5]. Some mixes such as a mitochondria nutrient mix [130] have been considered. Often mixes seem to be predicated on old ideas such as a combination of carotenoids, omega-3 fatty acids, and vitamin E which also may demonstrate a passing benefit [161]. The point of this work is to determine, or at least guess based on current knowledge and observations, a better cause to focus the response. Another work combined A,D,E,K and multiple B vitamins as well as probiotics with motivation being oxidative stress, inflammation, and dysbiosis [204]. However, its not clear what is causing all those things or why they matter. In particular, oxidative stress may be a signal for among other things copper deficient mitochondria. Too many to mention employ some combination including obscure or exotic herbs or ingredients [107] often for glib reasons. Some success has been observed in probiotic trials [168] which of course likely effect GI chemistry. Dietary patterns, similar to issues with "meals" discussed here, have also gotten some attention for empirical correlation with dementia such as a MIND pattern [47], Mediterranean and DASH [219], Japanese [234], and there are others [195] and other outcomes considered. A high sugar dietary pattern was analyzed with HR as a function of sugar intake showing an optimum but increasing beyond that [235]. This will be useful later if considered in the context of auto-brewery syndrome and methanol exposure. The partially beneficial ones of course suggest ingredients but not all components are likely to be beneficial.

I.8. Various Toxin Hypotheses

A variety of theories exist about cumulative exposure to environmental toxins such as "harmful metals, pesticides, agrochemicals, and air pollution" [56] or "heavy metals, flame retardants, and pesticide residue" [77]. Agent orange may produce some similar symptoms as AD [54]. This work will look mostly at toxics thought to be produced endogenously in the impaired stomach. The most prominent of these is methanol and its potential metabolites such as formaldehyde. Methanol has been well explored for cognitive impairment [123] as has formaldehyde [227]. These will be discussed in more detail as the current hypothesis is elaborated. However, a huge array of induced secondary metabolites are possible from any GI tract microbiome so its difficult to rule out anything. Some potentially important ones have been discussed [237]. Conversely, some marine dinoflagellates toxins have been considered for reduction in amyloid burden [26]. And its also worth mentioning that acetylcholinesterase inhibitors while usually considered toxic may be therapeutic [199].

II. THE PRESENT HYPOTHESIS

The thesis of this work is similar to some aspects of the gut based hypotheses except it directs attention to the likely underlying cause of dysbiosis or other phenomena linked to gut-brain-axis theories. Some works recognize an association between AD GI tract disorders but then tend to suggest things like, "antibiotic therapy, probiotics and prebiotics, and faecal transplantation are being considered as possible therapeutic options" [39] as in the dysbiosis theories.

A 2025 work did elaborate on the concept of pre-digested food for special populations such as the elderly with a detailed review of some issues [222] and it discusses some approaches such as fermentation that may be explored herein. However, it does not appear to address toxin production by the distressed GI tract.

As no particularly good suspect has emerged, it may be worth looking at all the data and trying to guess at age related vulnerabilities that cause infections to lead to dementia, digestive disturbances, and classic symptoms such as amyloid build ups. Nutritional defects make some sense as frailty, more than age per se, may be a correlate of age related diseases.

II.1. brain microbiome

In a prior work [137] I interpreted a brain microbiome result to describe some host factors that would explain differences in the most and least abundant organisms in AD and healthy brains post mortem. Microbiome studies on low-biomass samples have been criticized previously for susceptibility to various errors [113]. While the existence of a brain microbiome is controversial due to the expected low absolute abundance and opportunity for contamination, the analysis is somewhat robust in that contamination from other parts of the patient should still reflect differential body nutrient levels. One argument against contamination was similarity to an unrelated work on uterine microbiome and possible symbiotic role of these organisms but still a lot of coincidences can occur. This work generally pointed to deficiencies in lipophilic amino acids but also suggested additional methanol in the AD patients. One likely source for this is the GI tract due to changes in microbiome or initially changes in chemistry. Other toxins could associate with methanol production too and a good strategy may be to restore GI chemistry back to youthful state.

II.2. methanol and auto brewery syndrome

Methanol can be generated in the body from ethanol [57] while both ethanol and methanol circulate in a normal person at the .5-1.5mg/L level and may be produced from carbohydrate consumption [98]. Endogenous ethanol production in the stomach has been observed [203] and considered. One post mortem found high ethanol in the stomach attributed to *Candida glabrata* [120]. A recent review of auto-brewery syndrome suggested specific organisms may not be the issue as much as "...the use of medications, notably proton pump inhibitors and antibiotics. When combined with a Western diet rich in fat and simple sugars, these factors significantly contribute to altering the gut environment and subsequently, ethanol production." [150] although a list of ethanol producers has been created [146]. In one patient an oral glucose tolerance test achieved an ethanol concentration of 315mg/dL in the blood [193].

II.3. benzoate

Its worth mentioning the putative benefits of benzoates that have been explored for dementia and its role as a modulator of microbial growth (preservative). It may only function in cases where dementia would otherwise be dominated by methanol or some other fermentation related products.

Amino acids are thought to play a role in AD but several results suggest them to be unimportant largely probably due to glib or superficial interpretations.

Thinking aloud

move some of this to the specifics sections later

II.4. tryptophan

Tryptophan is a good suspect for many reasons but various supplementation trials have yet to show an interesting benefit in AD. In this work, the failures will be attributed to formulation, a need for other components of the limiting pathways, and failure to address any sources of toxins like methanol from the pathological GI chemistry which may exist.

Among treated metabolic syndrome patients, the Kynurenine to tryptophan ratio, as well as inverse C20:3 and lysine amounts, correlates with the risk of mild cognitive impairment [95]. This work also mentions patient heterogeneity which may be an important factor in testing nutritional interventions.

Tryptophan as a contributor to many diseases has been reviewed in several publications [171] [102].

A mixture of 9 essential amino acids including tryptophan given for 28 days failed to show any meaningful benefits [208] but this may be too little, too short a time, the formulation was not clear, and given together they can compete for entry into CNS. Other factors may be needed to get uptake and transport. A tryptophan dipeptide, a product of fermentation which may have improved absorption, did have some effects in mice [13]. At 25mg/kg BW for 12 weeks, elderly with mood disorders showed some improvements [43]/ although formulation was not clear. A variety of tryptophan derivatives have been explored such as 5-HTP [3] and indole 3-propionic acid [117] have also been explored.

Once nutrient issues occur, they can cause other problems such as transport reduction making simple supplementation less likely to work easily. One small study in rats found an age dependence of transport across the BBB both saturable and passive diffusion rates decreased in older animals [209]. This would suggest that at a constant blood level less enters the brain and supplementation with tryptophan may be futile. However, the transport restriction may be simply due to loss of other nutrients with age. Of particular interest was the 1991 observation that oleic acid can open the blood brain barrier [206] which has been observed to change with age and AD [182]. The quality of the BBB could then be responsible for many nutrient deficiencies in the brain but as with other structural components may be limited by nutrient uptake from the GI tract. Note too these issues are not mutually exclusive. Tryptophan occurs as a conserved amino acid in transporters and at membrane interfaces. Once tryptophan is depleted transport and barrier may suffer.

II.5. histidine

Another borderline amino acid may be histidine which is precursor to histamine. One study in mice claimed it improves response to chronic hypoperfusion [201] although data suggest small differences in quantifies explored. A study a few decades ago did find reduced regional histidine in post mortem AD brains [145]. At least one clinical trial in AD was considered, NCT06169826 <https://clinicaltrials.gov/study/NCT06169826> but apparently withdrawn.

It may provide protection against ischemia [127] and reduced oxygenation may be a factor in aging.

II.6. arginine

Oral arginine as recently described as suppressing amyloid pathology in animal models of AD [71].

II.7. glp-1

GLP-1's have finally been getting popular press attention due to related nutrient deficiencies such as scurvy [198]. In at least one case report, with no suggested GLP-1 usage, severe GI symptoms may occur from reduced retroperitoneal fat pad buffering the aorta and superior mesenteric artery combined with other problems like scurvy [8].

And indeed scurvy, another state creating "weak structural proteins"

Thinking aloud

my new catch phrase, probably including the collagen sca

has been noted in the literature in particular with colonic pseudoobstruction [59]. Although there has also been concern raised about under estimating the harm of malnutrition related to their use [64]. Other nutrients of concern include vitamin D, iron, calcium, protein, B-1 and B-12 [213]

Particular concerns have been raised about the hidden danger of lean muscle mass loss and resulting lack of resilience. [212]. The GLP-1 data may also motivate an interest. GLP-1's apparently are causing non-specific nutrient deficiencies maybe similar to problems common in old age. Recent clinical trials showed either no benefit in AD with the Novo Nordisk product [2]

Thinking aloud

does the author realize running against "other" antidiabetic drugs is misleading? ROFL

or a positive trial [62] appeared to be cherry picked. A Novo-Nordisk presentation on the results, https://sciencehub.novonordisk.com/content/dam/sciencehub/global/en/congresses-and-scientific-publications/congresses/ctad2025/johannsen1/sliders/Symposium_4_Cummings_Jeffrey_V2_OH.pdf does suggest largely no effect either good or bad although some of the side effects may be unpleasant. Its likely that "early stage AD" implies existence of important nutrient limitations have already been there for years so the differential impact, despite GI symptoms, may be minor.

A pooled analysis of 4 trials and 112 AD and MCI patients showed no significant outcomes differences and numerical harm well below significance [163].

The emergence of clinically important aged appearance (" Ozempic face") [159] [189] and concerns about neurological adverse events [41] or "brain fog"i anecdotes [210] are well known despite several works suggesting they should be brain beneficial

. There are a lot of lessons here but the immediate thing of relevance is that nutrient limitation that is can be "plausibly denied" appears to generate some symptoms of aging and more to the point frailty. Interestingly, at least one study found a correlation between perceived face age and dementia risk [225].

Evidence pointing towards a cognitive benefit for GLP-1's may be a bit misleading. In one case, they were run against sulfonylureas which have known relevant side effects. In other cases, words suggesting a cognitive benefit in actuality mean that some risk factor has been reduced without clear evidence of real clinical benefit against dementia.

II.8. race and sex

Women and blacks appear to be at higher risk of AD than European males. Factors unrelated to the disease per se may confound results but that is the case with even politically correct association studies. A small 1996 study found that blacks has a higher found higher pH and HCO₃ secretion as well as higher prevalence of Helicobacter pylori infection and gastritis [50]. Difference in "normal" protein digestion in healthy males and females appear to be known and have been considered already for a basis of "engineering foods for women" [121]. Note that this empirical difference does not rely on a specific reason although the chemistry may be informative for meal design. A small 1982 study of relatives found that older and female probands had lower acid output although some results depend on how the output is normalized [112]. One study comparing healthy 23-42 year olds with 44-71 found more acid secretion in the older group [78].

While one study did find differences in serum tyrosine levels between older people with and without AD [132] blood levels may vary due to transport issues.

One obvious issue with skin pigment is tyrosine metabolism. Both the depletion of tyrosine and generation of toxic analogs could be considered. It is possible that constitutive skin pigmentation (CSP) evolved with adaptive mechanisms that may be instructive to investigate. CSP evolution is usually described as an issue of sun exposure with damage and vitamin D synthesis being driving forces in tone [92].

While CSP may predispose to various maladies, it may also be easily treated if existing adaptations are not already active. Indeed, speculation on beneficial tyrosine analogs can not be dismissed.

Thinking aloud

goog ai went wild with "dementia skin pigmentation"

And melanin itself is not inert once formed especially in the presence of UV light [202]. A link between dementia and melanin has been considered by many authors. A study of several thousand Taiwanese demonstrated a hazard ratio of over 12 for AD in vitiligo sufferers versus control (HR of 5.3 for any dementia) [38]. Vitiligo is an autoimmune

disease making interpretation difficult but it does suggest an age related problem with melanin due to covert pathogen or defective products.

Fringe ideas in the mainstream literature include ability of melanin to transfer light energy to the cell for ATP creation and loss impairs function [18] . More plausibly, loss of something like tyrosine or copper limits pigment synthesis in some idiosyncratic manner or more to the point constitutive pigmentation requires resources that are no longer plentiful.

In zebrafish, there was an unexpected link between PSEN2 and pigmentation [81] of unknown significance.

Tyrosine anecdotes around menopause may be one indicator. While not considered essential as it can be derived from phenylalanine, it is possible that overall supply of the precursor and regulatory systems reduce tyrosine production pathologically. This is most likely during times of "stress" and shortages. Interestingly in connection with GLP-1 there is a tyrosine dipeptide apparently acting as a signalling molecule. Amino acids such as tyrosine may have limited availability in cell cultures and dipeptides have been explored [104] for many years now. Also of interest is the need to supplement tyrosine as a dipeptide to Chinese hamster ovary cells in bioreactors [217] with work showing possible pickiness of cells for uptake although translation to human GI tract remains to be done. Similar problems have been noted for tyrosine usage in humans from either IV injection or parenteral feeding [134]. Tyrosine-tyrosine peptide is considered a hormone and may be responsible for anorexia in urea cycle disorder patients [154].

Comparison to other racial groups does not appear informative as data are all complicated.

Some context for phenylalanine to tyrosine conversion as part of the larger protein turnover dynamics has been explored [76].

Indeed GLP-1 is often considered with PYY for understanding obesity signalling [55]. While its unclear if PYY can replace the GLP-1 drugs, if supplementation is attempted it will benefit a lot from recognizing the issues explored here. Dipeptides or reproduction of "best" GI chemistry may be more important than simply adding tyrosine to diet.

A variety of nutrient deficiencies have been observed to correlate with AD risk such as B,D, and E [45].

II.9. Copper

Copper has a lot of motivation behind it but its rich chemistry may make it one of the more difficult to study. Copper is difficult to characterize but some results may motivate its importance. Overall it appears that AD brains are copper deficient and well formulated supplementation may be beneficial but it can not be excluded yet that other nutrients are limiting its distribution. At least one case report exists of a defect in ceruloplasmin resulting in chorea and dementia with iron deposition that resolved somewhat with fresh frozen plasma [230]. As early as 2016, copper overload was suggested for regenerative medicine and boosting mitochondria performance [186].

Sometimes there is a tendency to think of local levels as a passive result of supply but often complicated signalling generates overall results. As early as 2009, the paradox of excess copper with amyloid beta coincident with tissue copper deficiency was recognized and explained to some degree [89]. In prior work with dogs, the inability of chelation to produce clinical benefit in animals with hepatic copper accumulation pointed to a bottleneck in transport rather than a global excess [138]. Also in the above work it was noted that "copper toxicity" varied widely with the background diet suggesting it has to do with chemical reactions in the meal. Rats raised on a copper deficient diet showed increases in adrenal dopamine-beta-monooxygenase mRNA, protein levels, and enzyme activity in decreasing order [175]. This suggests that feedback mechanisms are attempting to increase activity levels with greater transcription rates but unable to metallize the extra translation products. A 2017 mass spectrometry study on post mortem brains found overall decreased copper in AD brains similar to that in Menkes' Disease [224].

One interesting in vitro work was able to image intracellular Cu resolving oxidation states [196]. The work suggests amyloid beta can act similar to a siderophore and import copper, instead of iron, into the cell leading to death unless rescued with a chelator. The authors note the probe may perturb physiology and that Cu is released into the synaptic cleft. Taken together with existing literature, this tends to point to a bottleneck that may be partially neuron specific. Eliminating the excess copper may prevent death in vitro but may not restore normal brain function.

II.10. Vitamin K

For whatever reason, vitamin K is often ignored even though interactions with APOE and appearance in "healthy" foods motivates at least an association with health.

An early proposal for vitamin K deficiency was described in 2000 citing "regulation of sulfotransferase activity and the activity of a growth factor/tyrosine kinase receptor (Gas 6/Axl) " [6].

In the last few years many works have explored vitamin K in old age conditions [63] [103] including association between vitamin K intake and cognition [25]. As bleeding or microbleeding is an issue in AD, its worth noting that 82

percent of "chronic stroke subjects" were vitamin K intake deficient and excessive vitamin E may also be a common problem [79]. The above work also describes more direct effects of vitamin K in AD including interactions with amyloid beta. (there are works that concentrate on emerging problems with vitamin E supplementation too, for example [111])

II.11. NAD+

NAD+ deficiency has also been explored in AD [88] and other age related degenerative conditions [122]. Disturbances of NAD+ can influence trajectory of viruses such as Japanese Encephalitis Virus in mice and human cells [108].

II.12. Choline

Choline has been the topic of several recent works [53] [216] adding to a long history for it [215] [40] and related pathways [70] including cholinesterase inhibitors [20]. The 3xTg-AD mouse apparently has low levels of circulating choline [99] and that may be instructive to determine if the model genetics relate to this. While often related to acetylcholine, its important to remember its role in nutrient absorption.

II.13. Taurine

Taurine has been proposed as possible disease modifying supplement [87].

II.14. SMVT Substrates

Biotin and the SMVT substrates [190] have also come up in AD work and dementia more generally [192] [46] . Biotin is critical for many processes and can be obtained from digestion or intestinal microbe production [105]. A 2026 study of dietary biotin and AD risk combined with neuropathology results in biotin deficient mice suggested a biotin deficiency as a contributing factor to AD [233].

II.15. Thiamine

Thiamine is also being considered [65] and that too is through to have acid sensitive absorption [119]. A 2026 work identified a new mechanism by which it could mitigate problems with amyloid beta by suppressing HIF-1a [10]. A 2012 study did show some benefits from supplementation in the elderly but due to poor absorption parental dosing was mentioned [179]. It seems that many studies on isolated nutrients have come to similar conclusions but not attempted to think through cause and effect more generally. A 1988 study claimed to use a niacinamide placebo [23] which motivates another problem of an unintentionally active placebo. And in fact niacinamide has been discussed as a treatment too [115].

II.16. Lithium

Lithium has recently been rediscovered [14] although positive review articles date back to at least 2012 [66] or so [143] and its not clear what happened over the intervening decade.

In 1971, it was observed that lithium could impact carbohydrate generally and inositol status in the brain [7]. In another manuscript considering inositol as a dog supplement (unpublished result)

but as of 2005 it could not be determined if lithium or inositol was more directly causal to brain changes that matter in the clinic [82]. This is likely a recurring issue in Alzheimer's and medicine overall. Associations are difficult to map to an actionable cause and effect sequence that can be beneficially modified. It is the main theme of this work that the primary cause is well removed from these associations and the elephant is not apparent from the sum of its parts. It is likely that both inositol and lithium are impacted by some common cause and simply supplementing one or the other or both can make some improvements but is a deadend to a cure.

II.17. Bicarbonate, Chloride, Phosphate

There may be some intersection between detergent chemistry and conditions needed in the GI tract to absorb vitamins and retard toxin impact.

Thinking outloud

III. COMMON AGE-RELATED DIGESTION DEFECTS

To begin to make sense of the above and look towards the design of a better intervention, some knowledge of likely defects and resulting deficiencies needs to be established. Existing literature is a guide but like all published works needs to be considered in light of available data. Most works tend to minimize the importance of typical GI aging on health apparently mostly based on assertions and assumptions. Contradictory observations would need to be considered carefully and its likely intervention design will be somewhat empirical hoping to gain information as work continues. For example Chapter 8 in a 2014 book [109] suggests a large reserve capacity and no real effects absent disease with lowered uptake being a consequence of reduced protein mass. In what follows the "reduced protein mass" may be considered a problem in itself consequent to reduced nutrient uptake for reasons yet to be determined for this work. By 2025 the problem of protein uptake seems to be recognized [85] [178]. Cause and effect may be quite difficult to trace however. Consider the case of aging colonic muscles cells with reduced performance related to L-type Ca currents [32]. Resting $[Ca^{++}]$ is remarkably low with a large ratio to extracellular levels [110] and therefore sensitive to leaks. In fact as early as 2003 reduced response and intracellular accumulation were considered as a reason for "age-related learning deficits" [144]. Calcium leak may occur for several reasons [136] [139] and mutation related disease suggest problems with high-infidelity translation that could occur with amino acid imbalances.

By 2025, methods to simulate elderly digestion in vitro were discussed because age-related GI properties were thought to be important for health related outcomes in the growing elderly population [223].

Thinking outloud

probably a bad citation, low quality journal doi info missing lol [1]

In extreme cases such as GI resection, problems related to malnutrition such as increased abscess risk and reduced ability to produce collagen for wound healing and other needs [1] . ADAMTS2 upregulation in the case of AD was part of the motivation to create this work as it suggests a chronic problem in collagen quality.

By 1992, common themes in impairment reports were emerging. One review suggested decreased taste and hypochlorhydria with decreased uptake of B-12 and minerals but increased absorption of lipids [187]. A small 1997 study of independently living people over 65 found about 90 percent had basal pH under 3.5 [90] although there is no indication of how this compares to the young or very old.

A study of 79 healthy elderly people found some indications of lower acidity than young people but did not consider it relevant in most individuals at least for drug absorption [188].

The pancreas and intestines may also be effected. One review mentions decreases in calcium, zinc, and magnesium but not copper in old rats [86]. However, the pancreas and intestines must work with whatever the stomach produces with the meal. Earlier, I considered that lack of an early low pH step may produce more idiosyncratic results as some mineral can irreversibly precipitate [138].

Around 2004, it was thought that reserve capacity was sufficient although some reduced absorption of vitamins such as A,D,K, and B6 but tended to minimize significance for treating aging [60]. One problem however is that even today sufficient intake and uptake may not be clear and the impact of multiple "low levels" could invoke adaptive responses.

Today however protein deficiency appears to be reasonably well accepted as an issue with aging and its manifestation as sarcopenia [177]. What may be less apparent is any changes in quality such as due to translational infidelity or lack of "other stuff." Sarcopenia or frailty appear to be highly correlated with vulnerability to several "age related" conditions [228].

Further, the impact of drugs common in the elderly on nutrient status is becoming recognized [17].

Atrophic gastritis is one form of digestive problem. It may be silent, associated with non-acid reflux, and lead directly to deficiencies of B-12 and iron [183].

The list of effected nutrients may be much larger however due to simple lack of data. Vitamin C destruction, rather than simple malabsorption, may be significant with increased stomach pH as well as calcium salt dissolution and absorption rates [35]. Presumably other cations than iron and calcium are impacted with copper being an important candidate likely to be ignored. Some studies suggest elevated serum levels with H pylori positive gastritis [9] which may

be expected as Ceruloplasmin is an acute phase protein and blood levels may reflect an immediate stress. Generally malabsorption of copper or excess zinc was described in 2008 as uncommon but increasingly recognized [33].

IV. MEAL ENGINEERING CONSIDERATIONS

Based on the above, the goal then is to replace nutrients and remove toxics that are more common in old age. Return to a youthful GI tract may be a reasonable goal although it may be possible to do even better. Conceivably if nutrient uptake can overcome GI limitations some healing may be observed making possible a return to normal food for a while.

Nutrients need only be supplied over "relevant" time scales. This can vary a lot from nutrient to nutrient and allows better segregation and rotation than trying to get everything everyday. Incompatibilities include chemistry and competition for receptors or enzymes or transporters etc.

Not every meals needs to be the same and in fact currently with the dogs one meal contains most things while a second is largely reserved for copper.

IV.1. Reduced toxins : chloride and benzoate

IV.2. Solubility : chloride, protons, phosphate, emulsifiers

IV.3. Free nutrients

V. CANDIDATE INGREDIENTS

Candidates are drawn from prior thoughts on food and vitamin supplements along with the presumed GI disturbances common in old age.

Any foods or supplements reputed to improve some age related condition could be examined. In many cases the folklore appears to be a useful starting point as it may rationalize well with the above theory or be supported by in-house work here with dog feeding.

One recent work on pre-digested food for groups suc as the elderly gives list of some ideas [222].

Whey peptides improved some functions in healthy people thought to be due to Trp-Tyr peptides [114].

Fermented foods meet many of the criteria for obtaining nutrients with age impaired digestions. They tend to have low pH, although a group of alkaline fermentation products is known [164] , and can include pH as part of the definition, and essential molecules such as free amino acids and vitamin k. They seem to be associated with reduced dementia risk [173]

Olive oil has been an important part of in-house dog diets (manuscript in preparation) as well as being a highlighted component of the Mediterranean Diet. A variety of possible causal pathways could be imagined for it but here it is considered somewhat uniquely, along with lecithin and phosphorous sources, as an absorption enhancer. Its interesting to note, although of unknown significance, that yeast with a higher amount of membrane oleic acid have higher ethanol tolerance [231]. As part of a meal formulation it may help with uptake of lipophilic nutrients.

Its interesting to note that while choline chloride was being explored under non-physiological conditions (120 C) for delignification of biomass (something similar to digestion of natural foods) chloride was found to be the only important component with NaCl working just as well [200] despite all the interest in deep eutectic solvents etc. Chloride was also thought to be reasonable for copper extraction compared to other biocompatible chemistries [138].

2 related objectives are to improve chloride levels and reduce pH. In the results cited previously, acid came from citric acid and HCl supplement formulations. Formulations that use HCl as a raw component would require great care and may be avoided initially. Some soft drinks such as diet cola beverages contain phosphoric acid that may have some benefits. Chloride was largely from potassium chloride and again added HCl.

Title			
Ingredient	Amount	InCompatible	Remarks
Essential Amino Acids			
Essential Fats			
Essential Metals			
B-vitamins			
Misc vitamins			
Ion control			
Digestion Improvement			
Microbiology/Toxin Control			

TABLE I. Catagories are never clean since small molecules always do many things and duplicates will be included

VI. DEVELOPMENT PATHS

Since many candidate components are GRAS, its possible to outline informal or "crowd sourced" data from motivated parties such as caregivers. Concerns about malicious data due to various incentives as well as other issues would have to be considered in detail. I have been using MUQED daily to record dog diets and relate to outcomes. This helped to identify olive oil as an improtant component although it may have a long response time if other sources of oleic acid such as chicken are included in the diet. The added workflow just requires a few minutes after each meal or dosing to enter whatever was given and have it checked for basic syntax. I use "vi" although more sophisticated or voice entry may be easy to do. The current implementation of MUQED creates a small vocabulary of foods, supplements and outcomes as well as a simply syntax for entering amounts right after or even skeletons before consumption. As the current implementation is designed for just "a few" people or dogs, everything is done with text files locally although collation with a standard vocabulary could be done with better scaling implementations.

My in-house results with dogs does suggest some folk lore or anecdotes may have useful signals but will require more data to sort out and a MUQED-like approach may fill the void.

Even if this fails to mitigate disease, it may not be a frivolous effort especially if any trends are observed with prescription medicines.

VII. BIGGEST PROBLEMS

Thinking outloud

lot of issues here include off-target effects, accurate prescribing patterns, population differences between users and non uers, any real effect etc. AGA seems to be pushing this things probably get ad revenue lol

As proton concentration is thought to be a big issue, it may be surprising that chronic PPI use does not lead to obvious epidemic of AD. Although in older people they may not have much real impact as stomach acid is already likely to be low (if PPI's are effective at anything it is likely through some other mechanism. If you can't tolerate stomach acid you are going to be malnourished anyway - something is wrong so you may expect accurately prescribed PPI users to already be predisposed to dementia). For H pylori control, it turns out they likely prevent a bacterial pump rather than a host pump from functioning leading to reduction [22]. In general they lead to less healthy microbiome and over abundance of Streptococcus that may also produce symptoms in patients [152]. Its worth noting that some species can secrete virulence factors inducible by a return to low pH [58]. The PPI's may appear to relieve symptoms but allow the organisms to proliferate making the situation worse with a return to physiological acid levels. (lol).

Current evidence is conflicting even among older or sick people who may already have GI impairments minimizing any effect of PPI's. A 2023 study of a 1.98 million Danish cohort found an incidence of any dementia ratio between PPI users and non-users of 1.36 but reducing with age [174]. The American Gastroenterological Association appears to advance the position that this is unlikely. One post hoc analysis of the ASPREE aspirin trial involving people older than 65, found no association between PPI use and dementia [149]. Reported results are heavily corrected for various factors and the raw HR was not immediately obvious. It is possible the low dose aspirin has some impact but difficult to tell as presented.

Protons and chloride appear to be differentially regulated and PPI's do not stop normal chloride secretion [48] despite the fact they may be used to prevent excess fecal chloride excretion in abnormal cases [4]. There are several chloride transporters in the stomach including calcium activated [181] that may decouple from PPI's. The cystic fibrosis literature may provide some clues.

However, the theory here is a bit empirical and recognizes other factors than pH alone may be important. With a non-zero supply of most required molecules adaptive responses may be able to move the effects of the deficiencies

around making the clinical presentation difficult to trace to original problem.

Even if the theory is sound and eventually is proven useful as a starting point, social issues are a big problem. For whatever reason amyloid drugs appear to be getting a more favorable response than is supported by known data. Anything involving skin pigment will attract angry people.

VIII. CONCLUSIONS

The present work seeks to find a robust intvention against AD, specifically LOAD but more generally most age related conditions, by engineering meals to mitigate or fix GI impairments that can reduce nutrient uptake and increase toxin production. It suggests that the sum of all the partial theories can best be described by this common source of failure or it identified the proverbial elephant from distinct observations of the pieces. One non-specific symptom is likely weak structural proteins as amino acids and various factors will be deficient. While several paths for investigation are possible, informal approaches by motivated parties may help avoid some problems of the past efforts such as clinging to a familiar target instead of following the evolving data and avoiding ill posed questions such as the action of a single molecular entity on a diverse randomized population instead of the components for even a single cure specialized to even one person.

IX. SUPPLEMENTAL INFORMATION

Example of MUQED output on dogs over several years, <https://github.com/mmarchywka/dogdata> and some MUQED source code https://github.com/mmarchywka/muqed_util

IX.1. Computer Code

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Appendix A: Statement of Conflicts

No specific funding was used in this effort and there are no relationships with others that could create a conflict of interest. I would like to develop these ideas further and have obvious bias towards making them appear successful. Barbara Cade, the dog owner, has worked in the pet food industry but this does not likely create a conflict. We have no interest in the makers of any of the products named in this work.

Appendix B: About the Authors and Facility

This work was performed at a dog rescue run by Barbara Cade and housed in rural Georgia. The author of this report ,Mike Marchywka, has a background in electrical engineering and has done extensive research using free online literature sources. I hope to find additional people interested in critically examining the results and verify that they can be reproduced effectively to treat other dogs.

Appendix C: Symbols, Abbreviations and Colloquialisms

TERM definition and meaning

Appendix D: General caveats and disclaimer

This document was created in the hope it will be interesting to someone including me by providing information about some topic that may include personal experience or a literature review or description of a speculative theory or idea. There is no assurance that the content of this work will be useful for any particular purpose.

All statements in this document were true to the best of my knowledge at the time they were made and every attempt is made to assure they are not misleading or confusing. However, information provided by others and observations that can be manipulated by unknown causes ("gaslighting") may be misleading. Any use of this information should be preceded by validation including replication where feasible. Errors may enter into the final work at every step from conception and research to final editing.

Documents labelled "NOTES" or "not public" contain substantial informal or speculative content that may be terse and poorly edited or even sarcastic or profane. Documents labelled as "public" have generally been edited to be more coherent but probably have not been reviewed or proof read.

Generally non-public documents are labelled as such to avoid confusion and embarrassment and should be read with that understanding.

Appendix E: Citing this as a tech report or white paper

Note: This is mostly manually entered and not assured to be error free.

This is tech report MJM-2026-001.

Version	Date	Comments
0.01	2026-01-17	Create from empty.tex template
-	March 17, 2026	version 0.00 MJM-2026-001
1.0	20xx-xx-xx	First revision for distribution

Released versions,
build script needs to include empty releases.tex

Version	Date	URL

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pages={ 31}
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Supporting files. Note that some dates,sizes, and md5's will change as this is rebuilt.

This really needs to include the data analysis code but right now it is auto generated picking up things from prior build in many cases

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2703 Mar 13 19:53 comment.cut f995244b5e95cab8bb975bbef4509f63
30284 Mar 13 19:53 elephant.aux b027c475535263763a1b40f70bf98e65
103114 Mar 13 19:53 elephant.bbl 52329232367110269df8c4dc98d0ae68
1053952 Mar 13 19:53 elephant.bib 9588bc297c6091e5b56f35d651b804ff
2877 Mar 13 19:53 elephant.blg 70860d1546419a97f9082e0524451bba
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25709 Mar 13 19:53 elephant.flx 58e1a903ffb4fafde40a9108f5b80582
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51782 Mar 13 19:53 elephant.log 7d437a931ea95811db39809a9c58f8e8
0 Mar 13 19:53 elephantNotes.bib d41d8cd98f00b204e9800998ecf8427e
7124 Mar 13 19:53 elephant.out ce259f228ba57ef75deabdd2d5a73d77
443291 Mar 13 19:53 elephant.pdf ccac759ba5112d07b847724410199c55
80066 Mar 13 19:53 elephant.tex bf323202e1effdfc0bcd271936827233
3394 Mar 13 19:53 elephant.toc 9fd23ea0452dc7587ed33843fd275b30
49863 Jan 17 03:59 /home/documents/latex/bib/mjm_tr.bib e93b93c88b4601ca44b7886f378f254a
48988 Jul 1 2025 /home/documents/latex/bib/releases.bib f2fdf87a36feddcbbab0918dc2b00129
7331 Jan 24 2019 /home/documents/latex/pkg/fltpage.sty 73b3a2493ca297ef0d59d6c1b921684b
7434 Oct 21 1999 /home/documents/latex/pkg/lgrind.sty ea74beead1aa2b711ec2669ba60562c3
1069 Oct 15 2021 /home/documents/latex/share/includes/disclaimer-gaslight.tex 94142
b063984d082bfff3b400abe0fb
425 Oct 11 2020 /home/documents/latex/share/includes/disclaimer-status.tex b276f09e06a3a9114f927e4199f379f7
3927 May 2 2024 /home/documents/latex/share/includes/mjmaddbib.tex d58183f98615737b38f9d6e03fc064de
117 Oct 16 2023 /home/documents/latex/share/includes/mjmlistings.tex c1c3bd564b7fb321b4d8f223c7d3cde3
4867 Jun 23 2025 /home/documents/latex/share/includes/mycommands.tex d78deef4710745e9b4b29511fd3b610d
1031 Nov 8 07:26 /home/documents/latex/share/includes/myrevtexheaders.tex 79e40ba4aee901851cf9a06010aed8c2
2919 Jun 9 2024 /home/documents/latex/share/includes/myskeletonpackages.tex 08
a01c33af793bf936c6684d2e792c
1538 Aug 14 2021 /home/documents/latex/share/includes/recent_template.tex 49763d2c29f74e4b54fa53b25c2cc439
7217 May 15 2023 /home/documents/latex/share/overrides/mol2chemfig.sty 5aa81c197d462c2153a794ccee9881ef
928210 Mar 13 18:40 non_pmc_elephant.bib 06c8e9943d81241b67c82c5ae2c15f39
442 Jan 30 13:27 nutrient_table.tex 3232a0e067049091f2b7e4ba74158414
125331 Mar 5 12:39 pmc_elephant.bib f681d6a08caf9b4f73a03696988a5adc
413 Jan 17 03:59 releases.tex 56213b8bd7e164dd6e668b62f5acf868

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33495 Jan 30 2023 /usr/share/texlive/texmf-dist/bibtex/bst/urllibst/plainurl.bst 46
      cfb45e24a877059f9f7e142c7fc61c
165211 Jul 10 2025 /var/lib/texmf/fonts/map/pdftex/updmap/pdftex.map 6b08d096f3d0fe5ff6b266061a2b45e0
5712750 Jul 10 2025 /var/lib/texmf/web2c/luahbtex/lualatex.fmt a41ebf2b4a8fce76020b8e7c277eca3a
443291 Mar 13 19:53 elephant.pdf ccac759ba5112d07b847724410199c55
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